GIT surgery VII (Radiology)

إجمالي النقاط 5/29

0/1

Which is the most imaging modality that is used for a patient with perforated peptic ulcer disease?

- a. Abdominal ultra sound
- b. Supine x ray abdomen
- c. Gastrogaffin swallow
- d. Upright chest x ray
- e. CT scan abdomen

الإجابة الصحيحة

d. Upright chest x ray

0/1

ج Hydatid cyst in the liver best imaging except:

- US
- Radioisotopes
- CT
- MRCP (or ERCP)

الإجابة الصحيحة

Radioisotopes

1/··· In intestinal obstruction, radiography: *surg ✗

✗ Erect & supine X-ray ◉

ليس هناك أي إجابات صحيحة

0/1 * :Regarding solid mass calcifcation in abdominal X-ray, one is false ✗

✗ Uterine fibroids. ◉

autosplenectomy in sickle cell disease ○

Adrenal calcifications. ○

Porcelain gall bladder. ○

ovarian dermoid ○

الإجابة الصحيحة

ovarian dermoid ◉

1/1 Regarding hepatobiliary imaging which one is true *surg ✓

✓ Normal intrahepatic ducts can not be visualized ◉

0/1 :Rgarding gastric outlet obstruction, one is false

- ✗ resulting from mechanical obstruction of stomach emptying.
- duodenal or gastric peptic ulcers are the most common cause.
- in infants, pyloric stenosis is the commonest cause of gastric outlet obstruction.
- barium meal is better than ultrasound in the diagnosis of pyloric stenosis in infancy.
- adenocarcinoma and lymphoma are a recognized cause of gastric outlet obstruction.

الإجابة الصحيحة

barium meal is better than ultrasound in the diagnosis of pyloric stenosis in infancy.

التعليقات

Ultrasound has superseded the barium meal

1/1 :Regarding Benign And Malignant gastric Ulcers, one is false ✓

- ✓ in malignant ulcers, Ulcer project beyond the gastric contour (extraluminal). ◉
- Irregular and shallow ulcer crater in malignant ulcers. ○
- in benign ulcers , the mucosal folds are smooth thin and reach the margin of the ulcer. ○
- in benign ulcer, there is thin lucent line at the ulcer base called Hampton line. ○
- Carman meniscus sign seen in malignant gastric ulcers. ○

0/1 :regarding Boerhaave syndrome, one is false ✗

- ✗ Spontaneous perforation of esophagus due to a sudden increased intraluminal pressure. ◉
- Pneumomediastinum is one of the imaging features. ○
- cause Pleural effusion usually left sided. ○
- Rupture immediately above diaphragm usually on right anterolateral site in 90%. ○
- Treatment is with immediate thoracotomy. ○

الإجابة الصحيحة

- Rupture immediately above diaphragm usually on right anterolateral site in 90%. ◉

0/1 :Radiological Signs of Small Bowel Obstruction , one is false ✗

✗ Multiple dilated bowel loops. ◉

Centrally placed bowel loops. ○

volvulae conniventis which is complete circular luminal fold seen within dilated bowel loops. ○

diameter of bowel loops more than 5cm. ○

الإجابة الصحيحة

diameter of bowel loops more than 5cm. ◉

0/1 :regarding gastric carcinoma, one is true ✗

✗ At barium examination ,gastric carcinoma typically produces an regular filling defect. ◉

FDG-PET-CT have an important role in the primary staging of gastric cancer. ○

CT is the main imaging modality for the pre-operative staging of patients with gastric cancer. ○

Endoscopic ultrasound is useful in staging of advanced gastric cancer. ○

الإجابة الصحيحة

CT is the main imaging modality for the pre-operative staging of patients with gastric cancer. ◉

0/1

:Gastric lymphoma , one is false ✗

✗ The stomach is the most frequent site of lymphoma involving the gastrointestinal tract. ◉

Gastric fundus is the most common site of stomach that affected by lymphoma. ○

either as primary disease or by in^ltration from adjacent nodes(secondary). ○

There may be extensive, bulky lymphadenopathy adjacent to the tumor. ○

may mimic gastric carcinoma. ○

الإجابة الصحيحة

Gastric fundus is the most common site of stomach that affected by lymphoma. ◉

التعليقات

Antrum

0/1 :calcified solid mass in abdominal radiograph include all except

- Adrenal calci^cations. (selected)
- Uterine fibroids
- Spleen (autosplenectomy in sickle cell disease).
- Renal tuberculosis with autonephrectomy
- 'porcelain' gallbladder

الإجابة الصحيحة

Spleen (autosplenectomy in sickle cell disease).

0/1 Causes of Air in Portal Veins, one is false

- Ischemia. (selected)
- Infective/inflammation
- Asthma.
- biliary intervention.
- Neoplastic conditions.

الإجابة الصحيحة

biliary intervention.

0/1 :Signs of irresectability of pancreatic adenocarcinoma include all except

- presence of liver metastases. (selected)
- retroperitoneal invasion.
- tumor size more than 5cm.
- ascites.
- tumor encasement of arteries & veins.

الإجابة الصحيحة

tumor size more than 5cm.

0/1

:regarding colonic strictures, one is false

- irregular edge of stricture with shouldering seen in malignant stricture.
- stricture of diverticular disease almost always in ascending colon.
- stricture of TB and chrons disease tend to affect caecum.
- ischemic stricture mostly in area between splenic lexure and sigmoid colon .
- long smooth tapered stricture seen in benign lesion.

الإجابة الصحيحة

stricture of diverticular disease almost always in ascending colon.

التعليقات

Site of stricture can narrow the differential diagnosis .stricture of diverticular disease almost always in sigmoid colon

0/1 :The causes of dilatation of the colon include all except

- bowel Obstruction. (selected, incorrect)
- Paralytic ileus.
- Volvulus.
- chronic Ulcerative colitis.
- toxic dilatation.

الإجابة الصحيحة

chronic Ulcerative colitis.

0/1 :regarding acute appendicitis , one is false

- distended non-compressible appendix. (selected, incorrect)
- appendicolith may be seen.
- thickened edematous wall.
- diagnosis usually obvious clinically.
- perforated appendix usually result in pneumoperitoneum.

الإجابة الصحيحة

perforated appendix usually result in pneumoperitoneum.

0/1

:Regarding sprue, one is false

- Dilatation of small bowel is the most typical Pnding. (selected)
- bubbly duodenum (Nodular changes in duodenum).
- Reversal of jejunal and ileal fold patterns.
- Increased incidence of malignancy and aggressive lymphoma.
- Hyposecretion.

الإجابة الصحيحة

Hyposecretion.

0/1

:regarding Carcinoid tumors, one is false ✗

- ✗ in GIT ,the appendix is the most common site affected. ◉
- about two thirds of SB carcinoid are multiple. ○
- it virtually never occur in esophagus. ○
- patients with carcinoid syndrome usually have liver metastases. ○
- in the SB ,the distal ileum is the most common site. ○

الإجابة الصحيحة

about two thirds of SB carcinoid are multiple. ◉

التعليقات

Carcinoid tumors
.Carcinoid tumors arise from enterochromaffin cells
:Location
GI tract, 85%
Location: appendix, 50% > small bowel (33%), gastric, colon and rectum (2%); virtually •
never occur in esophagus
of small bowel carcinoids arise in distal ileum 90% •
;of small bowel carcinoids are multiple 30% •
of GI tract carcinoids spread to mesentery 40%-80%

1/1 :Regarding hepatic imaging, one is false ✓

- ✓ fatty liver appear echogenic on ultrasound and decreased attenuation in CT scan. ◉
- CT scan is modality of choice for demonstration of liver injuries. ○
- the normal biliary tree can be seen obviously in ultrasound. ○
- Hepatic hemangioma appear echogenic in ultrasound. ○
- few hepatic hydatid cysts show calciPed wall. ○

0/1 :regarding hepatic cirrhosis, one is false ✗

- ✗ decrease in size of right hepatic lobe on imaging. ◉
- Cirrhosis is the most common cause of portal hypertension. ○
- Portosystemic anastomoses are formed in lower end of esophagus. ○
- CT scan can detect early stage of cirrhosis. ○
- associated Signs of cirrhosis at CT & US are splenomegaly and ascites. ○

الإجابة الصحيحة

- CT scan can detect early stage of cirrhosis. ◉

0/1 :regarding chron's disease, one is false ✗

✗ Skip lesions are virtually diagnostic of Crohn’s disease. ◉

The rectum is often spared. ○

The deep ulceration in Crohn’s disease results in Pstulae and abscesses. ○

At an late stage, one of barium enema fiondings is shallow ulceration(aphthous ulcer). ○

Risk of colonic cancer is slightly increased in chrons disease. ○

الإجابة الصحيحة

At an late stage, one of barium enema fiondings is shallow ulceration(aphthous ulcer). ◉

0/1 :regarding colon pathologies ,one is true ✗

✗ Sessile polyp has more risk of malignancy than pedunculated polyp. ◉

anemia and weight loss are the common presenting features in Caecal carcinoma. ○

rectosigmoid carcinoma often appear large without constriction and obstruction. ○

in Hirschsprung’s disease, the rectum is grossly dilated at barium enema. ○

The diagnosis of intussusception in children is often made using barium enema. ○

الإجابة الصحيحة

anemia and weight loss are the common presenting features in Caecal carcinoma. ◉

0/1 **?Which of the following is true regarding the epidemiology of appendicitis**



1- The highest incidence of appendicitis is found in elderly persons.

2- Appendicitis is more common among women than men.

3- The peak incidence of appendicitis is during the late teen years.

4- Older individuals have a higher rate of perforation.

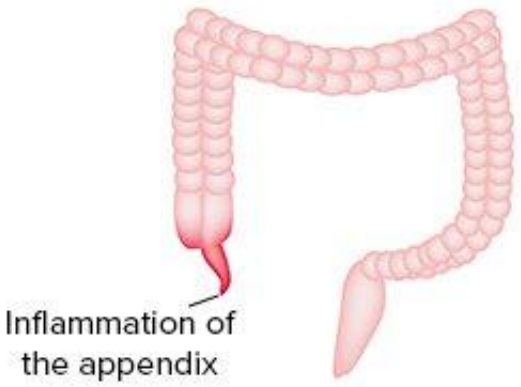
الإجابة الصحيحة

3- The peak incidence of appendicitis is during the late teen years.

التعليقات

In the past few years, a decrease in the frequency of appendicitis in Western countries has been reported, which may be related to improved dietary fiber intake. In fact, the higher incidence of appendicitis is believed to be related to poor fiber intake in such countries. There is a slight male preponderance of 3:2 in teenagers and young adults; in adults, the incidence of appendicitis is approximately 1.4 times greater in men than in women. The incidence of primary appendectomy is approximately equal in both sexes. The incidence of appendicitis gradually rises from birth, peaks in the late teen years, and gradually declines in the geriatric years. The mean age when appendicitis occurs in the pediatric population is 6-10 years. Lymphoid hyperplasia is observed more often among infants and adults and is responsible for the increased incidence of appendicitis in these age groups. Younger children have a higher rate of perforation, with reported rates of 50%-85%. This finding is explained by the frequency of nonspecific symptoms in young children with appendicitis. The median age at appendectomy is 22 years. Although rare, neonatal and even prenatal appendicitis have been reported. Clinicians must maintain a high index of .suspicion in all age groups

0/1 Which of the following statements is accurate regarding the presentation of appendicitis? ✗



✗ 1- The classic history of anorexia and periumbilical pain followed by nausea, right lower quadrant (RLQ) pain, and vomiting occurs in the vast majority of cases. ◉

2- Vomiting that occurs before pain is highly suggestive of appendicitis. ○

3- The presence of diarrhea or constipation rules out appendicitis. ○

4- Migration of pain to the RLQ is the most discriminating feature of the patient's history. ○

الإجابة الصحيحة

4- Migration of pain to the RLQ is the most discriminating feature of the patient's history. ◉

التعليقات

The classic history of anorexia and periumbilical pain followed by nausea, RLQ pain, and vomiting occurs in only 50% of cases. Nausea is present in 61%-92% of patients; anorexia is present in 74%-78% of patients. Neither finding differs statistically from findings in patients who present to the emergency department with other etiologies of abdominal pain.

In addition, when vomiting occurs, it nearly always follows the onset of pain. Vomiting that precedes pain is suggestive of intestinal obstruction, and the diagnosis of appendicitis should be reconsidered. Diarrhea or constipation is noted in as many as 18% of patients and should not be used to rule out appendicitis. The most common symptom of

appendicitis is abdominal pain. Typically, symptoms begin as periumbilical or epigastric pain migrating to the RLQ of the abdomen. This pain migration is the most discriminating feature of the patient's history, with a sensitivity and specificity of approximately 80%, a positive likelihood ratio of 3.18, and a negative likelihood ratio of 0.5. The pain migration is explained by the fact that the visceral peritoneum is innervated by thin, low-myelin nerve fibers, which conduct stimuli slowly (0.3 m/sec), thus rendering the brain incapable of accurately localizing the pain. When the appendix is inflamed, the brain is able to know only that such stimuli come from somewhere in the midgut, and the pain is felt around the umbilicus. As disease progresses and the full thickness of the appendix gets inflamed, the parietal peritoneum is affected. The latter is innervated by thick, myelin-rich fibers, which conduct stimuli very fast (120 m/sec). Thus, the brain is able to specifically pinpoint the anatomical origin of the pain in the RLQ. Patients usually lie down, flex their hips, and draw their knees up to reduce movement and to avoid worsening their pain. Later, the patient describes a worsening progressive pain, along with vomiting, nausea, and anorexia. .Usually, fever is not present at this stage

1/1 Which of the following characteristics is associated more commonly with ✓
?phlegmnous appendicitis than other stages of appendicitis



✓ 1- An inflamed or perforated appendix walled off by the adjacent greater omentum or small-bowel loops. ◉

2- Persisting tissue ischemia resulting in appendiceal infarction. ○

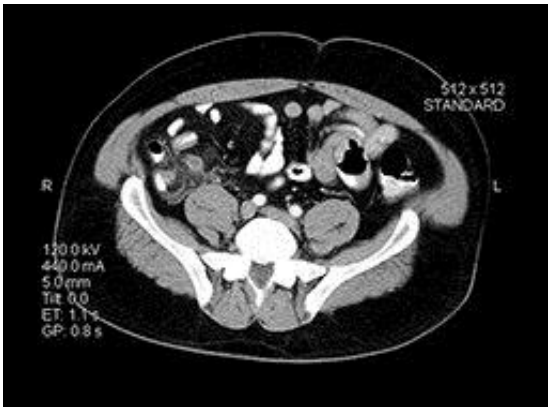
3- The presence of generalized or localized peritonitis. ○

4- Increasing intraluminal pressures that eventually exceed capillary perfusion pressure. ○

التعليقات

An inflamed or perforated appendix can be walled off by the adjacent greater omentum or small-bowel loops, resulting in phlegmonous appendicitis or focal abscess. In suppurative appendicitis, increasing intraluminal pressures eventually exceed capillary perfusion pressure, which is associated with obstructed lymphatic and venous drainage and allows bacterial and inflammatory fluid invasion of the tense appendiceal wall. Transmural spread of bacteria causes acute suppurative appendicitis. When the inflamed serosa of the appendix comes in contact with the parietal peritoneum, patients typically experience the classic shift of pain from the periumbilicus to the RLQ, which is continuous and more severe than the early visceral pain. In perforated appendicitis, persisting tissue ischemia results in appendiceal infarction and perforation. Perforation can cause localized or .generalized peritonitis

0/1 Which of the following statements is accurate regarding the workup of appendicitis? ✗



✗ 1- Abnormal urinalysis findings can be used to rule out appendicitis. ◉

2- CT has become the most important imaging study in evaluating patients with atypical presentations of appendicitis. ○

3- C-reactive protein (CRP) can be used to distinguish sites of infection. ○

4- Complete blood cell tests are specific for appendicitis. ○

الإجابة الصحيحة

2- CT has become the most important imaging study in evaluating patients with atypical presentations of appendicitis. ◉

التعليقات

CT with oral contrast medium or rectal Gastrografin enema has become the most important imaging study in the evaluation of patients with atypical presentations of appendicitis. Intravenous contrast is usually not necessary. The diagnosis of appendicitis should not be dismissed if urologic symptoms or abnormal urinalysis findings are present. CRP lacks specificity and cannot be used to distinguish between sites of infection. Complete blood cell tests are inexpensive, rapid, and widely available; however, the findings are nonspecific.

1/1 Which of the following statements is accurate about the treatment of appendicitis? ✓



✓ 1- For patients with a phlegmon, interval appendectomy can be performed 4-6 weeks after intravenous antibiotic therapy

2- Patients with larger, well-defined abscesses cannot be discharged while a catheter remains in place.

3- All patients with appendicitis, whether perforated or imperforated, should undergo immediate appendectomy.

4- Laparoscopic appendectomy is contraindicated in pediatric patients.

التعليقات

The following three treatment categories are generally recognized for an appendiceal mass:

- *Patients with a phlegmon or a small abscess: After intravenous antibiotic therapy, interval appendectomy can be performed 4-6 weeks later.*
- *Patients with a larger well-defined abscess: After percutaneous drainage with intravenous antibiotics, the patient can be discharged with the catheter in place. Interval appendectomy can be performed after the fistula is closed.*
- *Patients with a multicompartmental abscess: These patients require early surgical drainage. Historically, immediate (emergent) appendectomy was recommended for all patients with appendicitis, whether perforated or imperforated. More recent clinical experience suggests that patients who have perforated appendicitis with mild symptoms and localized abscess or phlegmon on CT can be initially treated with intravenous antibiotics and percutaneous or transrectal drainage. If the patient's symptoms, white blood cell count, and fever resolve satisfactorily, therapy can be changed to oral antibiotics and the patient can be discharged home. Then, delayed (interval) appendectomy can be performed 4-8 weeks later. According to the 2010 Society of American Gastrointestinal and Endoscopic Surgeons (SAGES) guideline, the indications for laparoscopic*

appendectomy are identical to those for open appendectomy. The 2010 SAGES guideline lists the following conditions as suitable for laparoscopic appendectomy:

- *Uncomplicated appendicitis*
- *Appendicitis in pediatric patients*
- *Suspected appendicitis in pregnant women.*

0/1

regarding temporary colostomy, all true except ✗

- ✗ defunctioning colostomy
- to relieve distal obstruction
- in high vesico-colic fistula
- to protect proximal anastomosis
- in emergency traumatic laceration of colon

الإجابة الصحيحة

- to protect proximal anastomosis

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